CLINICAL NOTE:

A 7-month-old infant brought to the pediatric outpatient clinic with a 3-day history of clear rhinorrhea, low-grade fever of 38.1 C, persistent hacking cough, and poor feeding.

PHYSICAL EXAMINATION:

Weight: 8.1 kg (50th percentile). Heart rate 142 bpm, respiratory rate 54 breaths per minute (tachypnea), SpO2 93% on room air. The infant is irritable but consolable. Examination of HEENT demonstrates copious clear nasal secretions with mild bilateral tympanic membrane erythema without purulent middle ear effusion. Chest examination reveals moderate subcostal and intercostal retractions, prominent nasal flaring, and diffuse bilateral expiratory wheezing with scattered coarse crackles throughout all lung zones.

DIAGNOSIS AND MANAGEMENT:

1. Acute viral bronchiolitis, presumed Respiratory Syncytial Virus (RSV).

2. Deep nasal suctioning performed in clinic utilizing olive-tip suction with immediate improvement in work of breathing.

3. Rapid viral nasal PCR swab obtained.

4. Instruct parents on aggressive home bulb-syringe nasal saline suctioning prior to feeds.

5. Emphasize small, frequent feedings to prevent dehydration. Return precautions given for worsening retractions or lethargy.